FREE PARENT GUIDE

Is it ADHD?

An early-signs, assessment and next-steps guide for parents

What ADHD really looks like in children and teens, how to tell it apart from "just a phase", and what to do next.

eMbrace Lives

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START HERE

Before you read on

If you are reading this, something about your child's attention, energy or behaviour has been on your mind. That is a caring, observant place to start from. This guide is meant to give you clarity, not a label.

This guide is for information, not diagnosis

Only a qualified clinician can diagnose ADHD, after a proper assessment. Use this guide to organise what you are noticing and to decide whether it is time to talk to a professional.

ADHD in 60 seconds

Attention-Deficit/Hyperactivity Disorder (ADHD) is a neurodevelopmental condition. It affects how the brain manages attention, activity levels and impulse control, mainly through the brain's self-regulation ("executive function") systems. It is one of the most common childhood neurodevelopmental conditions, affecting an estimated 5–7% of children worldwide, and it often continues into adulthood.

What it is

A difference in how the brain regulates attention, energy and impulses.

Present from early childhood and across settings: home, school, play.

Highly treatable with the right support.

What it is not

Not bad parenting.

Not laziness, defiance or a lack of intelligence.

Not caused by sugar, screens or "too much freedom", though routines matter.

Why early support helps

Fewer school struggles and confidence dips.

Less conflict at home.

Skills built early carry into the teen and adult years.

How to use this guide

- Read the three ADHD "presentations" and the age-by-age signs (pages 3–4).
- Tick the observation checklist and fill the one-week log (pages 5 and 10).
- Check the look-alikes (page 6) so you know what else could be going on.
- Bring your notes to a consultation. They are genuinely useful to the clinician.

UNDERSTANDING ADHD

The three ways ADHD can show up

ADHD does not look the same in every child. Clinicians describe three "presentations". A child can move between them over time.

Mostly inattentive

Daydreams, seems "switched off"

Loses things, forgets instructions

Struggles to finish tasks or homework

Easily distracted by small things

Often missed, especially in girls, because the child is quiet

Mostly hyperactive-impulsive

Fidgets, can't stay seated

Talks a lot, interrupts

Runs or climbs at the wrong times

Struggles to wait their turn

Acts first, thinks later

Combined

A mix of both sets of signs

The most commonly diagnosed presentation

Needs support for both focus and self-control

A note on girls and quieter children

ADHD in girls is diagnosed later on average, often in the teen years or adulthood. Girls are more likely to show the inattentive presentation, to "mask" with perfectionism or people-pleasing, and to be described as "dreamy" or "over-sensitive". If your daughter works twice as hard to keep up and is exhausted by it, it is worth a closer look.

What clinicians look for

For children, the usual diagnostic framework asks whether several signs of inattention and/or hyperactivity-impulsivity have been present for **at least 6 months**, started **before age 12**, show up in **two or more settings** (for example home and school), and are clearly getting in the way of learning, friendships or family life. No single sign on its own means ADHD.

SIGNS BY AGE

What ADHD can look like as children grow

Every child is different. Use these as a guide to patterns, not a scorecard.

Age group	What parents often notice
Preschool (3–5 years)	Constant motion; very hard to settle for stories or meals; frequent "near-miss" accidents; extreme tantrums that go beyond what other children of the same age show; trouble taking turns; difficulty following even simple two-step instructions; being asked to leave play groups or nursery.
Primary school (6–11 years)	Forgetting books or homework; incomplete work despite clear ability; blurting out answers; fidgeting or leaving the seat; rushing and careless mistakes; conflict over homework every evening; friendships strained by interrupting or bossiness; teacher notes such as "distracted", "talkative" or "capable of more".
Teens (12–17 years)	Chronic disorganisation and lateness; leaving work until the last minute; strong inner restlessness rather than visible hyperactivity; risk-taking or impulsive decisions; mood swings and low frustration tolerance; falling grades as demands increase; low self-esteem and anxiety underneath.

Signs that often travel with ADHD

- **Emotional intensity:** big reactions, quick frustration, hurt by criticism.
- **Time blindness:** "five minutes" and "one hour" feel the same.
- **Uneven attention:** can hyper-focus for hours on games or a favourite interest yet cannot start homework.
- **Sleep difficulties:** trouble winding down, late nights, hard mornings.

Parent tip

Notice *when* things go well, too. A child who can focus on a favourite activity for hours but not on homework is showing an attention-regulation difference, not an attitude problem.

SELF-CHECK

Parent observation checklist

Tick what you see often, meaning more than most children of the same age, for at least 6 months, in more than one setting.

Attention and focus

- [ ] Struggles to stay focused on tasks, play or reading
- [ ] Makes careless mistakes in schoolwork
- [ ] Seems not to listen when spoken to directly
- [ ] Starts tasks but doesn't finish them
- [ ] Has trouble organising belongings, time or homework
- [ ] Avoids or dislikes tasks needing sustained effort
- [ ] Loses things needed for daily life (bag, bottle, books)
- [ ] Is easily distracted and forgetful in daily routines

Hyperactivity and impulsivity

- [ ] Fidgets, taps or squirms when seated
- [ ] Leaves their seat when expected to stay put
- [ ] Runs, climbs or can't slow down in the wrong situations
- [ ] Finds it hard to play or relax quietly
- [ ] Talks excessively
- [ ] Blurts out answers or interrupts others
- [ ] Has trouble waiting for their turn

Impact in daily life

- [ ] Schoolwork or grades are affected
- [ ] Friendships or sibling relationships are strained
- [ ] Mornings, homework or bedtime are daily battles
- [ ] The child seems unhappy or low in confidence about it

READING YOUR RESULTS

What your ticks might mean

How many ticks	What to do
Fewer than 6 in attention or hyperactivity	Keep observing and use the one-week log. Try the home strategies on page 9. If concerns grow or school raises them, speak to a professional.
6 or more, and they have lasted 6+ months	Worth a professional conversation. Share your checklist and log with your paediatrician or a clinical psychologist.
6 or more, plus clear impact at school or home	A comprehensive assessment is recommended. Early support makes a real difference.

This is a screening aid, not a test. Many other things can look like ADHD.

Is it ADHD or something else?

Several conditions can look like ADHD, or exist alongside it. A good assessment looks at all of them.

Could also be…	Clues
Anxiety	Restless or distracted mainly when worried; avoids tasks out of fear of getting them wrong.
Learning difficulty (e.g. dyslexia)	Attention drops mostly during reading, writing or maths; bright in conversation but struggles on paper.
Autism spectrum	Differences in social communication, rigid routines or sensory sensitivities alongside attention concerns.
Sleep problems	Tired, irritable or "wired"; snoring or very late sleep onset.
Hearing or vision issues	Doesn't respond when called; squints or sits close to screens.
Stress, change or low mood	Sudden change after a move, loss or bullying; withdrawal alongside distraction.

ADHD frequently co-occurs with anxiety, learning difficulties and autism. If you suspect more than one, ask for an assessment that looks across all of them, rather than treating each in isolation.

MYTHS AND FACTS

Six things people get wrong about ADHD

The myth	The reality
"It's bad parenting."	ADHD is neurodevelopmental, with a strong genetic component. Parenting style does not cause it, though consistent routines and positive strategies help a lot.
"He's just lazy or not trying."	Children with ADHD often try harder than peers for the same result. Effort isn't the problem. Regulating attention is.
"They can focus on games, so it can't be ADHD."	ADHD is about regulating attention, not having none. High-interest, fast-feedback activities are easy. Routine tasks are not.
"They'll grow out of it."	Hyperactivity often softens, but attention and organisation difficulties commonly continue into adulthood without support.
"Girls don't get ADHD."	Girls are affected too, but are diagnosed later because their signs are more internal and quieter.
"A diagnosis will label my child."	A diagnosis opens doors to the right support, school accommodations and self-understanding. Not knowing is often harder.

Are you wondering about yourself, too?
ADHD runs in families. Many parents recognise themselves while reading about their child. Adults can be assessed and supported too. See embracelives.com/adhd/adult-adhd.

How to talk to your child about it

- **Keep it simple and positive:** "Your brain works like a race car with bicycle brakes. We're going to help you build stronger brakes."
- **Separate the child from the behaviour:** ADHD explains why some things are hard. It does not excuse hurting others or skipping effort altogether.
- **Name their strengths:** creativity, energy, curiosity, big-picture thinking and humour are common in children with ADHD.
- **Let them ask questions** and revisit the conversation as they grow.

WHAT TO EXPECT

What an ADHD assessment involves

A good assessment is a careful, collaborative process, not a single quick test or questionnaire. At eMbrace, it follows these steps.

1. **Initial consultation**
   You share your concerns, your child's history and what daily life looks like. No pressure and no judgement.
2. **Developmental and school history**
   Early milestones, medical and family history, sleep, behaviour and school reports.
3. **Rating scales from home and school**
   Standardised questionnaires such as the Conners-3, completed by parents and teachers, give a picture across settings.
4. **Direct work with your child**
   Observation and age-appropriate tasks. A cognitive assessment such as the WISC-V may be used where it adds clarity, for example to check for learning difficulties.
5. **Rule-outs and co-occurring conditions**
   Looking at anxiety, learning, language, sensory, sleep and other factors that can mimic or accompany ADHD.
6. **Feedback session and written report**
   A clear explanation of findings, a written report you can share with school, and a personalised plan.

Good to know

The number of sessions varies by child and by what needs to be explored. If medication is ever considered, it is a decision made together with a doctor (a psychiatrist or paediatrician). Many children do very well with therapy, parent training and school support alone.

After the assessment, support may include behavioural therapy and parent coaching, skills support for organisation and study habits, occupational therapy or speech therapy where relevant, and a school plan with accommodations.

WHAT HELPS

Practical strategies you can start this week

You do not need a diagnosis to try these. They help most children, and they are especially useful for children with attention difficulties.

At home

Predictable routines for mornings, homework and bedtime, with a visual checklist on the wall.

One instruction at a time. Get eye contact, say it simply, ask them to repeat it back.

Break tasks into chunks of 10–15 minutes with a short movement break.

Catch them being good. Specific, immediate praise: "You started without being reminded."

Protect sleep and movement. Fixed wind-down time, daily physical play, screens off before bed.

At school

Ask for **front-of-class seating**, away from distractions.

Request **written instructions** alongside spoken ones.

Agree on a **movement break** or a helper role.

Use a **home–school communication notebook** to track wins.

Ask about **extra time** and chunked tests where needed.

Remember

Children with ADHD are not choosing to be difficult. They are working with a brain that finds regulation harder. Your calm, consistent, encouraging response is the most powerful tool you have.

Seek help sooner if you notice

Safety concerns (running into roads, climbing dangerously); a sharp drop in school performance; school refusal or persistent low mood; frequent aggression or intense outbursts; talk of feeling "stupid" or "bad"; or sleep that stays very disrupted. These are reasons to book a consultation now rather than wait.

For a quick developmental check-in, try our free **Child Milestone Checker** at embracelives.com/resources/child-milestone-checker, and the **ADHD & Autism Screener** at embracelives.com/resources/adhd-autism-screener.

PRINT AND FILL IN

One-week observation log

For one week, jot down moments when attention, activity or impulse control got in the way, and also moments when your child did well. Bring this to your consultation.

Situation (e.g. homework, mealtime, school morning)	What I noticed	How often (daily / weekly)	Where (home / school / other)

Questions worth bringing to the appointment

- What are you seeing at home that you don't see at school, and the reverse?
- Has a teacher raised a concern? Can you share notes or report cards?
- How is sleep, appetite and mood?
- Any history of ADHD, learning difficulties or anxiety in the family?
- What is working well that we should build on?

NEXT STEP

Take the next step with eMbrace

Ready for clarity? Let's talk.

If your checklist and log show a pattern, the next step is a conversation with a clinician. At eMbrace, our RCI-certified clinical psychologists, speech and occupational therapists work together under one roof, in person at both centres or online.

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Support hours	Monday to Saturday, 10 AM to 6 PM IST
More on ADHD	embracelives.com/adhd/adhd
Adult ADHD	embracelives.com/adhd/adult-adhd

Important information

This guide is for general education only. It does not provide a diagnosis, medical advice or a substitute for professional assessment. If you are worried about your child's safety or wellbeing, contact a doctor or your nearest emergency service. Statistics are broad worldwide estimates and individual experience varies. © eMbrace, a unit of SMD Wellness. All rights reserved.